

TEE Procedure

Anesthesia : Topical spray xylocain 10%	Sedation : No	Duration : 15 min	Complication : None
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Patient Information	
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Name : SUHAICHI ORIN	Study Date : 03/01/2024	Height : 155cm	Diagnosis : HOCM, MR moderate
MRN : 2023540463	DOB : 17/07/1964	Weight : 89kg	Referring physician : dr. Dwita Ryan, SpJP
Age :	Gender : female	BSA : 1.87m²	Operator : dr. Estu Rudiktyo, SpJP (K)
First Write Time : 03/01/2024 16:14:06	Last Write Time : 05/01/2024 15:18:04	Blood Pressure : 163 / 88 mmHg	Medical Device : Vivid E95

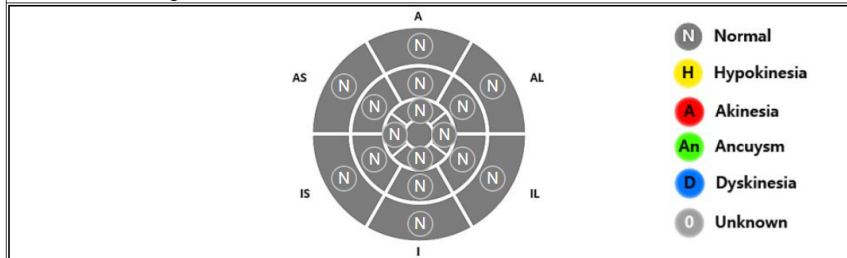
MMode / 2D Measurements & Calculations

IVSd : 13.6mm	Aor Diam : 36.6mm	EDV(teich) : 113.5ml	EF(Biplane) : 55%
IVSs : 27mm	LA dimen : 44.6mm	ESV(teich) : 12ml	EF(4ch) :
LVIDd : 49.1mm	LVOT diam :	EF : 89%	EF(2ch) :
LVIDs : 19.8mm	LA/Ao : 1.2mm	FS : 69.7%	IVC(Ins) :
LVPWd : 22.9mm	LAVI : 39.07ml/m²	TAPSE : 26mm	IVC(EXP) :
LVPWs : 29mm	LV mass Index : 226g/m²		

Doppler Measurements & Calculations

MV E vel : 90m/sec MV DT : 227msec MV A vel : 78m/sec E/A : 1.15	E/E (medial) : 15.61 E/E (lateral) : 15.11 E/E (average) : 15.36	MV V max : MV mean PG : MVA(plan) : cm²	MV VTI : MV PHT : MVA(VTI) : MVA by PHT :
AV V max : AV max PG : AV mean PG : AV VTI : AVA (VTI) : AVA(Vmax) :	AI PHT : AI(EROA) : LVOT VTI : Descending Aorta(EDV) : SV(LVOT) : AVA(plan) : cm²	TR V max(systolic) : TR max PG(systolic) : TV mean PG(diastolic) : TV VTI : PV Acc time :	

2D Wall-Motion Diagram ☐



Findings ☐

ECG Rhythm	ECG is sinus rhythm.
Study Quality	The quality of the study is adequate.
Left Ventricle	Left ventricle size is normal. There is asymmetric septal hypertrophy. RWT 0.9 ; LVMI 226 gr/m2. There is dynamic gradient at LVOT 68-95 mmHg. Global left ventricular systolic function is normal. LV Diastolic Function is indeterminate
Right Ventricle	Right ventricle is normal in size and function.
Left Atrium	Left atrium is dilated.
Right Atrium	Right atrial size is normal.
Interatrial Septum	Intact interatrial septum.
Interventricular septum	Intact interventricular septum.
Aortic Valve	The aortic valve is trileaflet. There is calcification at LCC. The aortic valve opens well. No aortic regurgitation is seen. There is no valvular aortic stenosis.
Mitral Valve	The mitral valve structure is normal. Mitral valve opens well. There is moderate mitral regurgitation due to systolic anterior motion of the AML (disrupting the normal coaptation of AML and PML). MR VCD 3.4 mm. Blunted S wave at pulmonary vein flow. There is no mitral stenosis.
Tricuspid Valve	Tricuspid valve is normal in structure and function. There is no tricuspid regurgitation.
Pulmonic Valve	Pulmonic valve appears normal. There is no pulmonary regurgitation.
Mass/Thromboses	There is no abnormal mass or thrombus.
Pericardium	Pericardium is normal.
Aorta	There is atherome plaque at descending aorta.

Conclusion

- Hypertrophic Cardiomyopathy (HCM) with significant dynamic gradient at LVOT
- Moderate MR due to systolic anterior motion of AML
- Good LV systolic function, LVEF 55%
- Good RV contractility
- Diastolic function is indeterminate
- Global normokinetics

Comments ☐

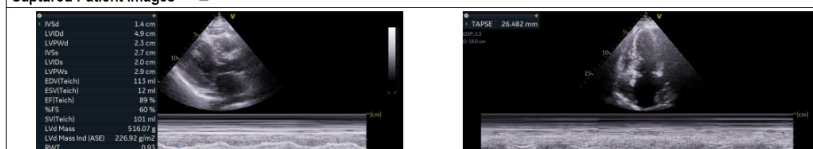
Operator : _____ Fellow : _____

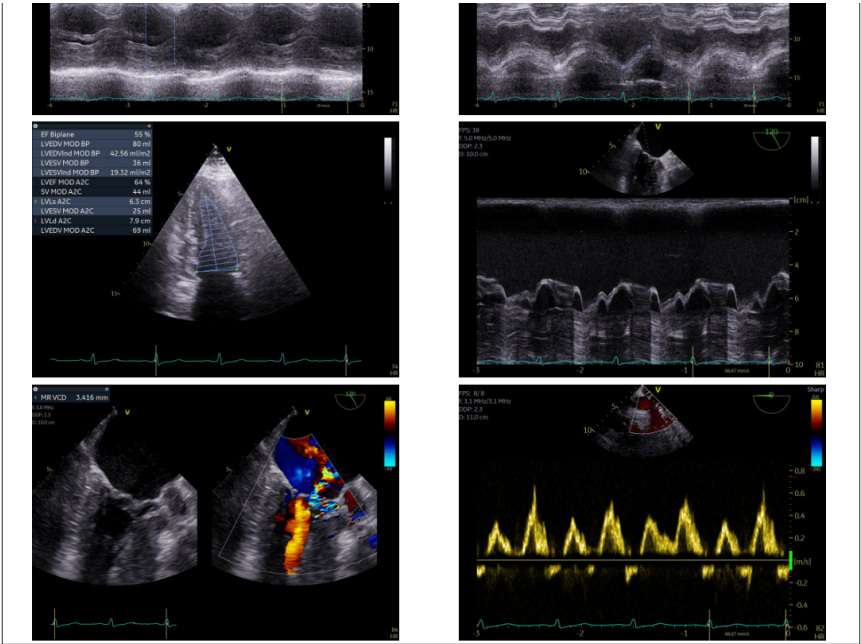
Supervisor Confirm : _____

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Captured Patient Images ☐





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